

YSP ALLOPURINOL TABLET

Allopurinol presents a new approach to the therapy of hyperuricemia associated with gout and other conditions. It is a potent xanthine oxidase inhibitor with the chemical formula 4-hydroxypyrazole (3,4-d) pyrimidine (HPP).

Ingredient(s):

Allopurinol Tablet :

Each tablet contains:

Allopurinol 100mg

Allopurinol Tablet 300mg :

Each tablet contains:

Allopurinol 300mg

Pharmacodynamics:

- Allopurinol acts on purine catabolism, without disrupting the biosynthesis of vital purines, by reducing the production of uric acid thus inhibiting the biochemical reaction immediately preceding its formation. Allopurinol reduces both the serum and urinary uric acid levels by inhibiting the formation of uric acid. It thereby avoids the hazard of hyperuricosuria in patients with gouty nephropathy or with a predisposition to the formation of uric acid stones.
- Allopurinol has produced a substantial reduction in serum and urinary uric acid levels in hitherto refractory patients even in the presence of renal damage marked enough to render uricosuric drugs virtually ineffective.
- Allopurinol is a structural analogue of the natural purine base, hypoxanthine. It is a potent inhibitor of xanthine oxidase, the enzyme responsible for the conversion of hypoxanthine to xanthine and of xanthine to uric acid, the end product of purine metabolism in man.
- Allopurinol also inhibits the enzymatic oxidation of mercaptopurine, the sulfur-containing analogue of hypoxanthine, to 6-thiouric acid. The oxidation, which is catalyzed by xanthine oxidase, inactivates mercaptopurine.

Pharmacokinetics:

$t_{1/2}$ = 18 ~ 30 hours. Metabolized by the liver and eliminated as Alloxanthine (Oxipurinol) by renal.

Indications:

Allopurinol is indicated for the main clinical manifestations of urate/uric acid deposition. It is also indicated for the management of 2,8-dihydroxyadenine (2,8-DHA) renal stone and recurrent mixed calcium oxalate renal stones.

Dosage and Administrations:

Dosage should be adjusted by monitoring serum and urinary urate concentrations. In general, adult: 100 - 200mg daily in mild conditions, up to 900mg in severe condition. Dose requirement in excess of 300mg should be administered in divided doses.

Elderly: The lowest dosage which produces satisfactory urate reduction should be used.

Severe renal insufficiency: < 100mg/day.

Renal dialysis: Allopurinol and its metabolites are removed by renal dialysis. If dialysis is required 2 - 3 times a week, consideration should be given to an alternative dosage schedule of 300 - 400mg allopurinol immediately after each dialysis with none in the interim.

Children < 15 years: 10 - 20mg/kg body weight/day or 100mg - 400mg daily.

Route of Administration:

To be taken orally.

Contraindications:

The drug should not be employed in patients with known intolerance of allopurinol. Pending further investigation, this drug is contraindicated for use in children with the exception of those with hyperuricemia secondary to malignancy. Allopurinol is contraindicated as a treatment for the acute attack of gout.

Precautions / Warnings:

- Prophylactic administration of colchicine is advisable during the early stages of therapy to prevent precipitate of acute attacks of gout.
- A fluid intake sufficient to yield a daily urinary output of at least 2 liters and the maintenance of a neutral or, preferably slightly alkaline urine are desirable to avoid the theoretic possibility of formation of xanthine calculi under the influence of allopurinol therapy, and to help prevent renal precipitation of urates in patients receiving concomitant uricosuric agents.
- Patients with impaired renal function should be carefully observed during the early stages of allopurinol administration and the drug withdrawn if increased abnormalities in renal function appear.
- It is recommended that periodic determination of liver and kidney function and complete blood counts be performed.
- Allopurinol should be discontinued at the first appearance of skin rash or other signs which may indicate an allergic reaction. Hypersensitivity to Allopurinol usually appears after some weeks of therapy, and more rarely immediately after beginning treatment. In some instances, a skin rash may be followed by more severe reactions such as exfoliative, urticarial and purpuric lesions as well as Stevens-Johnson syndrome, and / or generalised vasculitis, irreversible hepatotoxicity and even death.

- f) Use in pregnancy and lactation: There is inadequate evidence of safety of Allopurinol in human pregnancy. It should only be used in pregnancy when there is no safer alternative and when the disease itself carries risk for the mother or child. Allopurinol is excreted in human milk. There are no data concerning the effects of the drug on the breastfeed baby.

Drug Interaction:

1. Allopurinol may enhance cytotoxic effects of azathioprine and mercaptopurine with increased toxicity. These interactions are potentially hazardous and combined administration of the drugs involved should be avoided.
2. Concurrent use of Allopurinol with captopril may result in increased risk of toxicity, especially in renal impairment.
3. Allopurinol may possibly enhance effects of anticoagulants such as nicoumalone and warfarin.
4. Plasma-cyclosporin concentration possibly increased by Allopurinol and results in increased risk of nephrotoxicity.
5. The plasma half-life of adenine arabinoside is increased in the presence of allopurinol. When two products are used concomitantly, extra vigilance is necessary to recognise enhanced toxic effects.
6. Oxipurinol, the major metabolites of allopurinol and itself therapeutically active, is excreted by the kidney in a similar way to urate. Hence, drugs with uricosuric activity, eg. probenecid or large doses of salicylates may accelerate the excretion of oxipurinol. This may decrease the therapeutic activity of allopurinol, but the significance needs to be assessed in each case.
7. If allopurinol is given concomitantly with chlorpropamide when renal function is poor, there may be an increased risk of prolonged hypoglycaemic activity.
8. Allopurinol may inhibit hepatic oxidation of phenytoin, but the clinical significance has not been demonstrated.

Side Effect / Adverse Reactions:

Skin rash, nausea, vomiting, diarrhoea and intermittent abdominal pain, maculopapular rash, alkaline phosphate increases, SGOT/SGPT increases, fever/chills, arthralgia, cholestatic jaundice, eosinophilia, mild leucocytosis, leukopenia, ecchymosis, headache, myopathy, peripheral neuropathy, neuritis, paresthesia and somnolence has been reported.

Generalised Hypersensitivity: Allopurinol should be discontinued at the first appearance of skin rash or other signs which may indicate an allergic reaction. Hypersensitivity to allopurinol usually appears after some weeks of therapy, and more rarely immediately after the beginning of treatment. In rare instances, a skin rash may be followed by more severe reactions. eg. Stevens-Johnson syndrome and/or generalised vasculitis, irreversible hepatotoxicity and even death.

Blood and lymphatic System: Occasional reports have been received of thrombocytopenia, agranulocytosis and aplastic anaemia particularly in individuals with impaired renal function reinforcing the need for particular care in this group of patients.

Symptoms and Treatment For Overdosage, and Antidotes:

Symptoms and signs of overdosage include nausea, vomiting, diarrhoea and dizziness. Adequate hydration to maintain optimum diuresis facilitates excretion of allopurinol and its metabolites. Haemodialysis may be used if necessary.

Storage Conditions:

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

Shelf-life:

3 years from the date of manufacture.

Product description and Packing(s):

Allopurinol Tablet :

A yellow color round tablet with the diameter of 9.1mm, one side impressed with a "  " mark.

Plastic bottle of 100's and 200's

Blister packing of 10's x 10 and 10's x 100.

Allopurinol Tablet 300mg :

A white color round tablet, one side with a score.

Blister packing of 10's x 10 and 10's x 100.

TABLET YSP ALLOPURINOL

Bahan Aktif:

Tablet Allopurinol:

Setiap tablet mengandungi:

Allopurinol 100mg

Tablet Allopurinol 300mg:

Setiap tablet mengandungi :

Allopurinol 300mg

Indikasi:

Allopurinol digunakan untuk rawatan manifestasi utama klinikal pembentukan batu urate / asid urik. Ia juga digunakan untuk rawatan batu ginjal 2,8-dihydroxyadenine (2,8-DHA) dan pengulangan campuran batu ginjal kalsium oxalate.

Dos:

Dos boleh diubah-suai dengan mengawasi kepekatan urate dalam serum dan urinari.

Secara umum, dewasa: 100 - 200mg sehari bagi keadaan ringan, dos maksima ialah 900mg sehari untuk keadaan tenat. Dos melebihi 300mg perlu dibagi kepada beberapa kali sehari.

Orang tua: dos terendah yang dapat mengawal asid uric dengan memuaskan disyorkan.

Kerosakan ginjal yang serius: < 100mg sehari

Dialisis ginjal: Dos 300 - 400mg Allopurinol setiap kali lepas dialisis.

Kanak - kanak 15 tahun: 10 - 20mg / kg berat badan sehari atau 100mg - 400mg sehari.



Manufacturer and Product Registration Holder:

Y.S.P. INDUSTRIES (M) SDN. BHD. (199001001034)

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